National Cancer Control Programme (NCCP) 00561 · clinician draft

Carboplatin + etoposide + chest radiotherapy

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · Carboplatin + etoposide

Active medicines: Carboplatin + Etoposide

Emetogenic risk: moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally	Before carboplatinAUC3 Adjust for QT/individual risk.
Dexamethasone	8 mg orally on the platinum day and next2 days	Reference moderate-risk course
Metoclopramide	10 mg orally up to three times daily as required	Rescue use as prescribed

Additional phase-specific support

- Multiday systemic anticancer therapy (SACT): reconcile antiemetics for each day's medicines and the two days after; one combined prescription is required. Do not add independent overlapping courses.
- CarboplatinAUC3 is moderate risk here; do not import the AUC≥4 high-risk course simply because the drug is carboplatin.
- Coordinate radiation-related nausea, swallowing, nutrition, skin care and respiratory assessment with radiation oncology.
- No automatic routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is specified in these National Cancer Control Programme (NCCP) regimens; consider individual risk and local policy.
- Review etoposide interactions, live vaccines, nephrotoxic/ototoxic medicines and any duplicate QT-prolonging antiemetics.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00561 v5 · pNCCP 00561 supportive care; NCIS V8 Table 1 and multiday section · NCCP NCIS antiemetics V8

Sources checked 2026-11-02 · SACTCheck v0.84.30 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Days 2–5 · etoposide only

Active medicines: Etoposide

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg orally once daily	Before the etoposide-only dose Reconcile any existing delayed-emesis5-HT3 plan; do not duplicate.
Metoclopramide	10 mg orally up to three times daily as required	Rescue use as prescribed Prescriber confirms suitability and duration.

Additional phase-specific support

- Multiday systemic anticancer therapy (SACT): reconcile antiemetics for each day's medicines and the two days after; one combined prescription is required. Do not add independent overlapping courses.
- CarboplatinAUC3 is moderate risk here; do not import the AUC≥4 high-risk course simply because the drug is carboplatin.
- Coordinate radiation-related nausea, swallowing, nutrition, skin care and respiratory assessment with radiation oncology.
- No automatic routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is specified in these National Cancer Control Programme (NCCP) regimens; consider individual risk and local policy.
- Review etoposide interactions, live vaccines, nephrotoxic/ototoxic medicines and any duplicate QT-prolonging antiemetics.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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Day 8 · Carboplatin only

Active medicines: Carboplatin

Emetogenic risk: moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally	Before carboplatinAUC3 Adjust for QT/individual risk.
Dexamethasone	8 mg orally on the platinum day and next2 days	Reference moderate-risk course
Metoclopramide	10 mg orally up to three times daily as required	Rescue use as prescribed

Additional phase-specific support

- Multiday systemic anticancer therapy (SACT): reconcile antiemetics for each day's medicines and the two days after; one combined prescription is required. Do not add independent overlapping courses.
- CarboplatinAUC3 is moderate risk here; do not import the AUC≥4 high-risk course simply because the drug is carboplatin.
- Coordinate radiation-related nausea, swallowing, nutrition, skin care and respiratory assessment with radiation oncology.
- No automatic routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is specified in these National Cancer Control Programme (NCCP) regimens; consider individual risk and local policy.
- Review etoposide interactions, live vaccines, nephrotoxic/ototoxic medicines and any duplicate QT-prolonging antiemetics.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00561 v5 · pNCCP 00561 supportive care; NCIS V8 Table 1 and multiday section · NCCP NCIS antiemetics V8

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